Ibà — Fever-Triage Copilot

Clinical Logic Review

This document explains, in plain language, every deterministic clinical rule Ibà applies — the parts of the system that do not depend on the AI model's judgement. It is written for clinical review, not for engineers: there is no code, only what each rule looks for, what it forces, and where it comes from in the national or WHO guidelines.

Reviewer: Dr. Oluba O. Amos, PharmD (@AmazingAmos)

Date reviewed: _____

How to mark a rule: circle Agree or Disagree under each rule below, and if Disagree, write the correction you'd make on the line provided. Ibà is built to escalate rather than under-triage wherever a guideline doesn't fully settle a question — flag anywhere that trade-off looks wrong for real PHC use, not only outright errors.

CORE PRINCIPLE: rules can raise the triage level — never lower it.

Every rule in this document computes a triage "floor." Ibà's final triage level is always the more urgent of (a) the AI model's own reasoning and (b) this floor — never less urgent than the floor, regardless of what the AI concludes. If the AI step fails outright (no result at all), the case fails safe to Refer now rather than defaulting to a green result. In short: the rules in this document are a safety net the AI cannot talk its way under, only above.

Agree / Disagree *(core principle)* **Correction:** _____

1. Danger signs

Ibà checks the worker's own words — in English or Nigerian Pidgin — against ten fixed danger signs, independently of whatever the AI model concludes. These checks run twice: once immediately on the raw text (so a clear danger sign can show "Refer now" within about a second, before any AI call completes), and again after the AI's own reasoning, in case the AI's structured read of the case catches a sign the text rules missed.

Any single sign detected **forces Refer now** on its own — no combination is required. The design deliberately favours catching a sign over precision: a missed danger sign is treated as a safety bug; an unnecessary referral is not.

A stated negation ("no convulsions," "not bleeding") suppresses a text-rule match for that phrase, but never suppresses a sign the AI model itself flagged from its own reading of the case.

Sign	Plain meaning	English phrasing	Pidgin phrasing	Forces	Citation
Convulsions	The patient is having, or has just had, a fit / seizure.	convulsions; seizure(s); fits; fitting; jerking movements	(same words are used in Pidgin; no distinct local phrasing beyond the above)	**Refer now**	WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8); also a WHO Guidelines for Malaria, severe malaria criteria (p.212) feature ("multiple convulsions")
Unable to drink or breastfeed	The patient cannot take fluids or breastmilk at all, or is drinking/feeding poorly.	unable to / can't / refuses to / stopped drinking, breastfeeding, sucking, feeding; not drinking; drinking poorly; poor oral intake	e no fit drink/suck; e no gree drink; e no dey drink; e no wan drink/suck/chop	**Refer now**	WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8)
Vomiting everything	Everything the patient eats or drinks comes back up.	vomiting / throwing up everything, all, anything; can't keep anything down; persistent, repeated, incessant or uncontrollable vomiting	everything wey e chop/drink, e dey vomit/throw am	**Refer now**	WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8)
Lethargy or unconsciousness	The patient is abnormally sleepy, hard to wake, confused, or	lethargic; unconscious; unrousable; can't wake/rouse; drowsy;	e no dey respond; e no gree wake; e dey sleep anyhow	**Refer now**	WHO IMCI Chart Booklet (2014), general

Sign	Plain meaning	English phrasing	Pidgin phrasing	Forces	Citation
	unresponsive.	comatose; unresponsive; not responding; confused; floppy; passed out; sleeping too much/all day; reduced level of consciousness			danger signs (p.2, p.8); also a WHO Guidelines for Malaria, severe malaria criteria (p.212) feature ("impaired consciousness")
Abnormal bleeding	Bleeding from somewhere it shouldn't — nose, gums, vomit, stool, injection site.	bleeding; nosebleed; haemorrhage; epistaxis; haematemesis; blood from nose/mouth/gums/eyes/ears/ vagina/injection site/cannula/drip site/vomit; coughing or spitting blood; bloody vomit, urine, sputum or spit	blood dey comot/commot (from wherever)	**Refer now**	WHO Guidelines for Malaria, severe malaria criteria (p.212) ("bleeding")
Jaundice	Yellowing of the eyes, skin or body.	jaundice; icterus; yellow eyes/skin/body/sclera/palms; eyes turning yellow	eye don yellow; body dey yellow	**Refer now**	WHO Guidelines for Malaria, severe malaria criteria (p.212) ("jaundice")
Respiratory distress	Difficult, laboured, or unusually fast breathing; chest pulling in; noisy breathing.	difficulty/trouble/ struggling/laboured/hard breathing; breathless; shortness of breath; chest indrawing; grunting; gasping; nasal flaring; deep breathing, whether or not also described as fast or rapid (fast breathing ALONE, without “deep,” does not match this sign — see note below); stridor; panting; dyspnoea	breath dey cut; e dey pant	**Refer now**	WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8) and a WHO Guidelines for Malaria, severe malaria criteria (p.212) feature ("respiratory distress / acidosis")
Severe pallor	Unusually pale palms, eyes, lips, tongue or body — a sign of severe anaemia.	severe / very / extreme / marked / profound pallor or paleness; severe anaemia; palms/eyes/lips/tongue/body turning white	no blood for (im/her) body	**Refer now**	WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8) and a WHO Guidelines for Malaria, severe malaria criteria (p.212) feature ("severe anaemia")
Dark (cola-coloured) urine	Urine that is dark, black, brown, cola- or tea-coloured — a sign of haemoglobinuria.	dark/black/brown/cola/ coke/tea/red(-coloured) urine, piss or pee; haemoglobinuria; blackwater	piss/urine dey black/dark	**Refer now**	WHO Guidelines for Malaria, severe malaria criteria (p.212) ("haemoglobinuria / dark urine")
Prostration (unable to sit/stand)	The patient is too weak to sit, stand or walk unaided.	prostrate(d)/prostration; unable / too weak to sit, stand or walk	e no fit waka/sidon/stand; im body weak well well	**Refer now**	WHO Guidelines for Malaria, severe malaria criteria (p.212) ("prostration")

Beyond this list, Ibà also treats the following as the same signs above, not as separate rules: persistent/repeated vomiting and confusion/drowsiness/a “floppy” infant are folded into “Vomiting everything” and “Lethargy or unconsciousness” respectively. “Fast breathing” alone is deliberately not treated as a danger sign (WHO IMCI classifies it as pneumonia, a treatable condition, not a general danger sign) — chest indrawing, grunting, laboured breathing and stridor do still trigger “Respiratory distress.” Verified against the matching rule (2026-10-10): the pattern requires the word “deep” to be present — “breathing fast” or “fast breathing” on its own, with no other sign, does not match.

Agree / Disagree *(the ten-sign list and the fast-breathing exclusion)* **Correction:**

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1a. Severe dehydration (two-of-four rule)

Added 2026-09-28, after a live case (watery stool, vomiting, no fever, sunken eyes) was wrongly returned as Treat & monitor with “no danger signs detected.” WHO IMCI's “Classify Dehydration” table and the NCDC cholera guideline independently define the same four-sign Severe Dehydration picture:

- Lethargic or unconscious
- Sunken eyes
- Not able to drink, or drinking poorly
- Skin pinch goes back very slowly (≥2 seconds)

Two or more of these four → **Refer now** for IV rehydration (WHO's “Plan C”) — this fires regardless of whether the patient has a fever, because dehydration is a fluid-loss emergency, not a febrile-illness finding. Two of the four signs (“not able to drink/drinking poorly” and “lethargic or unconscious”) are already separate danger signs above and trigger Refer now on their own; this rule adds the other two (sunken eyes; slow skin pinch) and combines all four, so a patient with, say, sunken eyes plus a slow skin pinch is caught even though neither sign alone would trigger.

Citations: WHO IMCI Chart Booklet, “Classify Dehydration” table; NCDC Guidelines for Acute Watery Diarrhoea (Cholera) Outbreak, “Severe dehydration: two or more of the following signs.”

“No urine output” is also checked for and counted as a fifth possible sign, because it was specifically asked for and is a clinically recognised sign of severe volume depletion — but, checked directly against both indexed documents, neither WHO IMCI nor the NCDC cholera guideline actually names it among their dehydration criteria. It is detected, but the citation above is to the shared four-sign definition, not to this fifth sign specifically. This is also flagged as an open question below.

Agree / Disagree *(the two-of-four dehydration rule, including the no-urine-output gap)* **Correction:**

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1b. Suspected Lassa fever

Based on the NCDC National Guideline for Lassa Fever Case Management (2018), §1.1.2 Suspected Case definition: fever for 3–21 days (if a temperature was actually measured, it must read ≥38 °C; an unmeasured temperature does not rule the case out) together with one or more of:

- Vomiting — "vomit*", "throw up"
- Diarrhoea — "diarrhoea", stool running/watery, "running stomach"
- Sore throat — "sore throat", "throat dey pain"
- Myalgia — muscle pain/ache, "body pain/ache"
- Generalised weakness — "(general/body) weak*"
- Abnormal bleeding
- Abdominal pain — "stomach pain/ache", "belle dey pain"

The index of suspicion is also raised, independently of the symptom list above, by:

- No response to a course of antimalarial and/or antibiotic treatment within 48–72 hours (e.g. "took Coartem, no change")
- Possible contact with a known/suspected Lassa case, or with rats/rodents, their droppings or urine
- Abnormal bleeding (one of the ten danger signs above)

How the triage level is set

Situation	Trigger	Forces	Citation
Live outbreak signal active in the patient's own state	Full case definition met, OR fever ≥3 days with no treatment response, OR abnormal bleeding	**Refer now**	NCDC Lassa Guideline §1.1.2; live signal via Tavily
No live signal, but the state	Full case definition met	**Refer now**	NCDC Lassa Guideline

Situation	Trigger	Forces	Citation
is a high-burden Lassa state (baseline)			§1.1.2
Same, high-burden state (baseline)	Partial features only: fever ≥3 days with no treatment response or possible contact, but none of the seven listed symptoms	**Refer within 24h**	NCDC Lassa Guideline §1.1.2 (partial)
State is “reported” tier only (not high-burden), no live signal	—	Does not escalate from this rule	See §4 Endemicity

Whenever Lassa is suspected at any tier above, Ibà also adds this fixed reminder to the actions, following NCDC 2018 §2.1.1: “Keep the patient in a separate holding area, use infection prevention measures (gloves; avoid contact with blood and body fluids), notify the LGA disease surveillance officer, and refer for Lassa testing.”

Agree / Disagree *(the Lassa suspicion rule and its live/baseline tiers)* **Correction:**

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2. Dosing — Artemether-Lumefantrine (AL)

Ibà's AI model never generates a drug dose, under any circumstance. Every dose shown to a health worker comes only from the fixed lookup table below.

Source: NMEP National Guidelines for Diagnosis and Treatment of Malaria (4th Edition, May 2020), §4.2.1 Artemether-Lumefantrine, Table 4-4 “Dosage Regimens for Artemether-Lumefantrine Treatment” (printed page 10; PDF page 24).

Verified against the printed Table 4-4 by Daniel Jemiri, 2026-09-28: all four weight bands and their boundaries, and every tablet count across all three strengths (including every NA cell), were checked against the national guideline document itself, not transcribed from memory.

Strength	5 to under 15 kg	15 to under 25 kg	25 to under 35 kg	35 kg and above
20/120 mg	1 tablet per dose	2 tablets per dose	3 tablets per dose	4 tablets per dose
40/240 mg	*NA — not suitable for this band*	1 tablet per dose	*NA — not suitable for this band*	2 tablets per dose
80/480 mg	*NA — not suitable for this band*	*NA — not suitable for this band*	*NA — not suitable for this band*	1 tablet per dose

Schedule: twice daily for 3 days (6 doses in total). NMEP's own text does not give an hour-by-hour breakdown (there is no "0 h / 8 h" schedule anywhere in the source document) — that level of detail is a WHO convention, not something NMEP itself states, and Ibà never attributes it to this citation.

Tablet splitting: “Tablet splitting is not recommended” (Table 4-4's own caption).

With food: absorption is enhanced by fatty meals — give after a meal, or with a tablespoon of milk.

What an “NA” cell means

An NA cell means that specific tablet strength is not suitable for that weight band. Ibà never borrows a tablet count from a different strength to fill an NA cell — if a strength has no dose for the patient's weight, Ibà shows a note to use a different strength instead, never a guessed number.

When no dose is shown at all

- The patient is not both RDT-positive and Treat & monitor (e.g. any Referred case) — no table dose is attached; pre-referral dosing is not yet in the table.
- Weight is not recorded — Ibà shows “Weigh the patient: the antimalarial dose depends on the weight band” instead of a dose.
- Weight is under 5 kg — below the whole table; Ibà shows “consult or refer; no table dose” rather than guessing downward.

Agree / Disagree *(the AL dose table, the NA-cell rule, and the no-dose conditions)* **Correction:**

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3. Follow-up questions and advice logic

3a. When Ibà asks a follow-up question

Three fields are always treated as critical: the patient's age, how many days the fever has lasted, and the malaria RDT result. A fourth field — whether antimalarials/antibiotics were tried and whether the fever responded — becomes critical only once the fever has lasted 3 or more days and that information hasn't already been given or inferred from the text.

If any critical field is missing after reading the worker's text, and the worker hasn't already answered a follow-up or asked Ibà to skip questions, Ibà asks — in priority order: fever duration, RDT result, treatment response, then age — up to two questions per round, in either English or Nigerian Pidgin depending on the language the worker wrote in.

Field	Question (English)	Question (Pidgin)
age_years	How old is the patient?	How old the patient be?
fever_days	How many days has the fever lasted?	How many days the body don dey hot?
rdt_result	What was the malaria RDT result: positive, negative, or not done?	Wetin malaria RDT show: positive, negative, or dem never do am?
treatment_response	Has the patient taken antimalarials or antibiotics for this fever? Did the fever get better?	The patient don take malaria drug or antibiotic for this fever? The fever don reduce?

Agree / Disagree *(which fields are critical, and the question order)* **Correction:**

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3b. What a green “Treat & monitor” result always includes

Whenever Ibà's final decision is Treat & monitor, the worker is always given a fixed, guideline-sourced set of safety-net advice — regardless of what the AI model itself generated — unless the case is explicitly afebrile (see 3c below):

- “Review in 3 days if the fever persists. Return immediately if any danger sign appears: convulsions, unable to drink, vomiting everything, very sleepy or unconscious, bleeding, yellow eyes or dark urine.” — WHO IMCI Chart Booklet p.8
- If the RDT was negative: “Do not give antimalarials: the malaria test is negative. Look for another cause of fever,” plus “If the fever persists, refer for further tests (for example typhoid testing or a full blood count)… Refer for assessment if fever is present every day for more than 7 days.” — WHO IMCI p.8 (“Malaria test NEGATIVE”; “fever >7 days, refer”) and the WHO Guidelines for Malaria test-before-treat statement
- If the RDT was not done (or the result is unknown): “Do a malaria RDT before giving any antimalarial (test before treating).” — WHO Guidelines for Malaria, test-before-treat good-practice statement

If the RDT was positive, neither of the two RDT-specific lines above is shown; the patient instead receives the dosing-table advice from §2.

3c. Recent fix: fever-path advice no longer leaks onto afebrile cases

Fixed 2026-09-28, alongside the dehydration rule above, after the same live miss: an afebrile patient (“no fever” explicitly stated) was still being told to “review in 3 days if the fever persists” and to have a malaria RDT done — advice that makes no sense when there is no fever to begin with. All of §3b above is now skipped entirely whenever the worker's case explicitly states zero days of fever. An unknown fever duration (simply not mentioned) still gets the usual advice — only an explicit “no fever” suppresses it. This only affects the Treat & monitor advice text in §3b; it has no effect on Refer now or Refer within 24h results, and no effect on whether a danger sign or the dehydration rule fires (those already fire regardless of fever, per §1a).

Agree / Disagree *(the always-included Treat & monitor advice, and the afebrile-case fix)* **Correction:**

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4. Endemicity — two-tier baseline per disease

Independently of any live outbreak search, Ibà keeps a static baseline list per disease, sourced from the most recent NCDC situation report available for each (checked 2026-09-28). Each disease's states are split into two tiers:

High burden — the states a disease is most active in right now, by the cited report's own numbers. These states are allowed to drive an escalation rule (for Lassa, this is the baseline branch of the Lassa suspicion rule in §1b).

Reported — states the same report names as having had cases, but not among the highest-burden ones. These states inform the differential and outbreak context shown to the worker, but never raise the triage level on their own, however the case otherwise presents.

A baseline entry's report date is shown to the worker as "older surveillance" once it is more than roughly 6 months old. That label is purely informational — it never changes whether a state can escalate; only the high-burden/reported tier above decides that.

Agree / Disagree *(the high-burden vs. reported distinction, and that reported states never escalate alone)* **Correction:** _____

4a. Lassa fever

Source: NCDC Lassa Fever Situation Report, Week 36, 2026, report date 2026-09-06, p.1.

HIGH BURDEN		
Ondo	**Escalates (rule may fire)**	34% of 2026 confirmed cases
Bauchi	**Escalates (rule may fire)**	24% of 2026 confirmed cases
Taraba	**Escalates (rule may fire)**	12% of 2026 confirmed cases
Edo	**Escalates (rule may fire)**	11% of 2026 confirmed cases
Benue	**Escalates (rule may fire)**	6% of 2026 confirmed cases
REPORTED (differential only — never escalates alone)		
Kogi	Differential only	New confirmed cases in week 36 itself; not yet high-burden
Ebonyi	Differential only	Carried over from the previous provisional list — confirmed: false (neither confirmed nor ruled out by this report; NCDC's full 24-state list is a map image, not machine-readable text)

24 states recorded ≥1 confirmed case in 2026 in total; only the states this report names in text are listed here.

Agree / Disagree *(the Lassa fever state list)* **Correction:** _____

4b. Cerebrospinal meningitis (CSM)

Source: NCDC Cerebrospinal Meningitis Situation Report, Week 14, 2025 (cumulative epi week 40 2024 – 14 2025), report date 2025-04-06, p.1.

HIGH BURDEN		
Sokoto	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
Kebbi	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
Zamfara	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
Katsina	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
Kano	**Escalates (rule**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's

	may fire)	24-state list
Jigawa	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
Yobe	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
Borno	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
Bauchi	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
Gombe	**Escalates (rule may fire)**	Meningitis-belt state (recurring high burden, dry season); confirmed in NCDC's 24-state list
REPORTED (differential only — never escalates alone)		
Adamawa	Differential only	≥1 suspected case all season; not part of the classic belt
Akwa-Ibom	Differential only	≥1 suspected case all season; not part of the classic belt
Anambra	Differential only	≥1 suspected case all season; not part of the classic belt
Bayelsa	Differential only	≥1 suspected case all season; not part of the classic belt
Benue	Differential only	≥1 suspected case all season; not part of the classic belt
Ebonyi	Differential only	≥1 suspected case all season; not part of the classic belt
Ekiti	Differential only	≥1 suspected case all season; not part of the classic belt
FCT	Differential only	≥1 suspected case all season; not part of the classic belt
Kaduna	Differential only	≥1 suspected case all season; not part of the classic belt
Niger	Differential only	≥1 suspected case all season; not part of the classic belt
Ondo	Differential only	≥1 suspected case all season; not part of the classic belt
Osun	Differential only	≥1 suspected case all season; not part of the classic belt
Oyo	Differential only	≥1 suspected case all season; not part of the classic belt
Plateau	Differential only	≥1 suspected case all season; not part of the classic belt

This is the newest CSM situation report NCDC has published as of 2026-09-28 — CSM reporting on ncdc.gov.ng has not been updated in over a year.

Agree / Disagree *(the Cerebrospinal meningitis (CSM) state list)* **Correction:** _____

4c. Cholera

Source: NCDC Cholera Situation Report, Week 9, 2025, report date 2025-03-02, p.6.

HIGH BURDEN		
Abia	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Akwa-Ibom	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Bauchi	**Escalates (rule**	Marked "Active" — reported cases in the most recent 3 weeks

	may fire)	
Bayelsa	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Borno	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Delta	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Imo	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Kebbi	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Kogi	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Lagos	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Nasarawa	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Osun	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Rivers	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
Taraba	**Escalates (rule may fire)**	Marked "Active" — reported cases in the most recent 3 weeks
REPORTED (differential only — never escalates alone)		
Adamawa	Differential only	Reported cases earlier in the season; not Active as of this report
Benue	Differential only	Reported cases earlier in the season; not Active as of this report
Ekiti	Differential only	Reported cases earlier in the season; not Active as of this report
Enugu	Differential only	Reported cases earlier in the season; not Active as of this report
Gombe	Differential only	Reported cases earlier in the season; not Active as of this report
Katsina	Differential only	Reported cases earlier in the season; not Active as of this report
Niger	Differential only	Reported cases earlier in the season; not Active as of this report
Ondo	Differential only	Reported cases earlier in the season; not Active as of this report
Oyo	Differential only	Reported cases earlier in the season; not Active as of this report
Sokoto	Differential only	Reported cases earlier in the season; not Active as of this report
Yobe	Differential only	Reported cases earlier in the season; not Active as of this report

Replaces the previous list entirely — Zamfara and Kano do not appear anywhere in this report and have been removed. Newest cholera sitrep as of 2026-09-28.

Agree / Disagree *(the Cholera state list)* **Correction:**

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Appendix: data/endemicity.yaml (verbatim)

The exact source file behind §4, included in full as plain text below, exactly as it is stored and used by the running system (no reformatting).

```
# Static endemicity baseline: used on every case, with live outbreak signals (Tavily) on top.
# If live data is unavailable, triage still knows that e.g. Ondo is a Lassa hotspot.
#
# Sourced against current NCDC situation reports (2026-09-28). Two tiers per disease:
#   high_burden - the states a live outbreak escalation rule (or, for Lassa, the baseline
#                 escalation branch of lassa_assessment) is allowed to act on.
#   reported    - states the cited report names, but not among its highest-burden ones.
#                 These inform the differential only; they never raise the triage level on
#                 their own (see backend/app/rules/danger_signs.py::endemic_baseline).
# A `reported` entry may set `confirmed: false` when it is carried over from an earlier list
# that the current report neither confirms nor rules out (its own data isn't machine-readable
# text - see Lassa/Ebonyi below); everything else is confirmed by the cited report itself.
#
# `report_date` is the end of the cited report's reporting period; the UI and reasoning
# prompt label a baseline "older surveillance" once report_date is more than ~6 months old
# (this never gates escalation - only the high_burden/reported tier does that).
#
# `anchor`: (doc_id, phrase) in an indexed guideline that supports using place of residence
# as a risk factor; resolved to a chunk ID at runtime for citation. Separate from `report`/
# `epi_week`/`page`, which cite the NCDC situation report that names the states themselves.

diseases:
  lassa:
    disease: Lassa fever
    verified: true
    report: NCDC Lassa Fever Situation Report
    epi_week: "Week 36, 2026"
    report_date: 2026-09-06 # end of the reporting period (31 Aug - 6 Sep 2026)
    page: 1
    source: >-
      NCDC Lassa Fever Situation Report, Epi Week 36 2026 (31 Aug-6 Sep 2026), p.1.
      high_burden: the 5 states accounting for 87% of 2026 confirmed cases (Ondo 34%,
      Bauchi 24%, Taraba 12%, Edo 11%, Benue 6%). reported: Kogi had new confirmed cases in
      week 36 itself, so it is named but not (yet) high-burden. Ebonyi is carried over from
      the previous provisional list: neither confirmed nor ruled out by this report, since
      NCDC's full 24-state list for 2026 is a map/figure image, not machine-readable text
      (confirmed: false). 24 states recorded >= 1 confirmed case in 2026 in total; only the
      states this report actually names in text are listed here.
    peak_months: [11, 12, 1, 2, 3, 4]
    anchor: [ncdc-lassa-advisory-2026, "particularly in endemic and high-burden states"]
    high_burden: [Ondo, Edo, Bauchi, Taraba, Benue]
    reported:
      - Kogi
      - state: Ebonyi
        confirmed: false

  csm:
    disease: Cerebrospinal meningitis
    verified: true
    report: NCDC Cerebrospinal Meningitis Situation Report
    epi_week: "Week 14, 2025 (cumulative Epi week 40 2024 - 14 2025)"
    report_date: 2025-04-06 # end of the reporting period (31 Mar - 6 Apr 2025)
    page: 1
    source: >-
      NCDC Cerebrospinal Meningitis Situation Report, Epi Week 14 2025 (cumulative Epi week
      40 2024 - 14 2025 season), p.1: "Twenty-four (24) States reported suspected CSM cases".
      high_burden: our existing 10-state meningitis-belt list (recurring high burden,
      dry-season north) - all 10 are confirmed in NCDC's 24-state list. reported: the other
      14 states NCDC's report names, which had >= 1 suspected case reported all season but
      are not part of the classic belt. This is the newest CSM sitrep NCDC has published as
      of 2026-09-28 (checked directly against the site's own listing) - CSM reporting on
      ncdc.gov.ng has not been updated in over a year.
    peak_months: [12, 1, 2, 3, 4, 5, 6]
```

```
    anchor: null
    high_burden: [Sokoto, Kebbi, Zamfara, Katsina, Kano, Jigawa, Yobe, Borno, Bauchi, Gombe]
    reported:
      - Adamawa
      - Akwa-Ibom
      - Anambra
      - Bayelsa
      - Benue
      - Ebonyi
      - Ekiti
      - FCT
      - Kaduna
      - Niger
      - Ondo
      - Osun
      - Oyo
      - Plateau

  cholera:
    disease: Cholera
    verified: true
    report: NCDC Cholera Situation Report
    epi_week: "Week 9, 2025"
    report_date: 2025-03-02 # report's own "as of 2nd March 2025"
    page: 6
    source: >-
      NCDC Cholera Situation Report, Epi Week 9 2025 (3 Feb-2 Mar 2025), state table p.6
      (cumulative summary p.2): 25 states reported cases since the start of the season.
      high_burden: the 14 states the table marks "Active" (reported cases in the most
      recent 3 weeks). reported: the other 11, which reported cases earlier in the season
      but were not Active as of this report. Replaces the previous list entirely: Zamfara
      and Kano do not appear anywhere in this report and have been removed. This is the
      newest cholera sitrep NCDC has published as of 2026-09-28 (checked directly against
      the site's own listing) - cholera reporting on ncdc.gov.ng has not been updated in
      over a year.
    peak_months: [6, 7, 8, 9, 10]
    anchor: null
    high_burden:
      [Abia, Akwa-Ibom, Bauchi, Bayelsa, Borno, Delta, Imo, Kebbi, Kogi, Lagos, Nasarawa, Osun,
Rivers, Taraba]
    reported:
      [Adamawa, Benue, Ekiti, Enugu, Gombe, Katsina, Niger, Ondo, Oyo, Sokoto, Yobe]
```

Sign-off

Section	Reviewer	Date	Decision / changes
Core principle			
1. Danger signs			
2. Dosing			
3. Follow-up & advice			
4. Endemicity			